



## PATIENT GUIDE

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# CATARACT

Understanding, managing and treating:  
A complete guide for patients of Samyak Drishti Eye Clinic



### Contact

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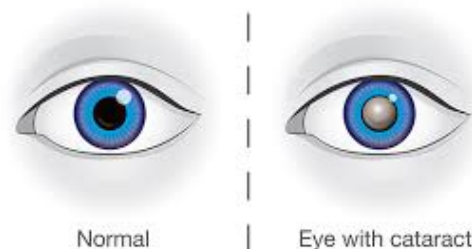
[www.samyakdrishti.com](http://www.samyakdrishti.com)

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## WHAT IS CATARCT?

The natural lens of your eye is normally clear, allowing light to pass through and focus sharply on the retina. With age, when the lens begins to become cloudy or opaque, it is called a cataract.

It is NOT a film or growth on the eye – it is a change within the lens itself. As the cataract progresses, it scatters incoming light instead of focusing it cleanly on the retina, leading to progressively blurred or hazy vision.



### Did you know?

Cataracts are the leading cause of preventable blindness worldwide. In India alone, more than 3 million cataract surgeries are performed every year, making it one of the most common (and most successful) surgical procedures in medicine.

## SYMPTOMS OF CATARACTS

Cataracts develop slowly and painlessly. Early changes may cause little or no visual disturbance, but over time you may notice:

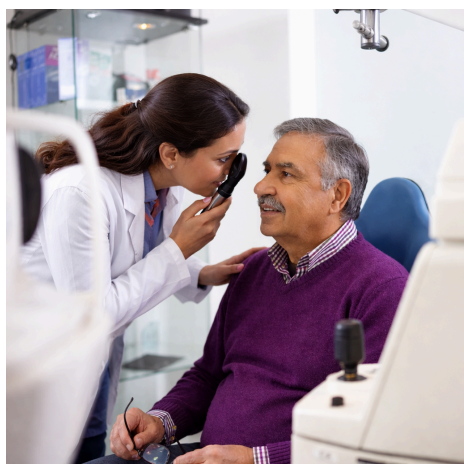
- Blurred, hazy or dim vision as though looking through a frosted or fogged-up window
- Increased glare and sensitivity to light, particularly in bright sunlight or from oncoming headlights at night
- Difficulty reading or using a screen, even with adequate lighting
- Fading or yellowing of colours
- Double vision or ghost images in one eye
- Frequent changes are needed in your spectacle prescription
- Difficulty recognizing the facial expressions of people near you



**Note:** If any of these symptoms are affecting your daily life: work, driving, or reading, speak to Dr Sujal or Dr Manisha Shah. Early assessment gives you the widest range of treatment options.

## DIAGNOSING CATARACTS

At Samyak Drishti, a thorough eye examination is performed to assess your cataract and overall eye health.



This typically includes:

**Visual acuity test:** Measures how clearly you see at different distances.

**Slit-lamp examination:** Allows the doctor to examine the front of the eye, including the lens, under magnification.

**Intraocular pressure (IOP) measurement:** Rules out glaucoma.

**Dilated retina assessment:** Eye drops widen the pupil to allow the doctor to assess the the retina fully before surgery is planned.

**Biometry:** Precise non-contact measurements to calculate the ideal intraocular lens (IOL) power

### For premium cataract surgery

- Anterior segment tomography – High-resolution scans to analyse your cornea, iris, angles,
- Retinal OCT – High-resolution scans to analyse the central part of the retina for suitability for presbyopia correcting IOLs

### Why Us

Premium IOL outcomes depend on precise measurements. Our in-house diagnostic suite, including Pentacam tomography and corneal biomechanics, ensures exceptional accuracy.

## When Should You Have Cataract Surgery?

In the early stages of cataract, a change in your spectacle prescription may temporarily improve vision. Surgery is considered when your vision affects your daily activities or lifestyle or when further glasses changes are no longer adequate.

There is no single objective threshold that determines exactly when cataract surgery must be done. The decision is ultimately yours.

Here are some important considerations:

| Factor           | What It Means                                                                                                                                                            |
|------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Early surgery    | Technically simpler procedure, faster recovery, wider IOL choices                                                                                                        |
| Waiting too long | A very dense ('mature') cataract is harder to remove and may slow recovery. It also carries greater risk                                                                 |
| Active lifestyle | If you drive, work, or use screens regularly, earlier surgery may be better                                                                                              |
| Fear of surgery  | Delaying due to anxiety often makes the cataract worse.                                                                                                                  |
| Refractive goal  | If you wish to significantly reduce dependence on spectacles, earlier surgery (PreLEx – Presbyopic Lens Exchange) may be considered even before vision is badly affected |

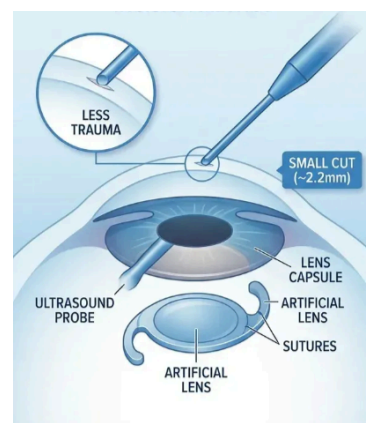
***Cataracts cannot be treated with eye drops, tablets, or any non-surgical therapy. Surgery is the only effective cure.***

## YOUR CATARACT SURGERY OPTIONS

Samyak Drishti offers the full spectrum of modern cataract surgery techniques. During your consultation, Dr. Sujal or Dr. Manisha will recommend the most suitable approach based on your cataract density, eye health, vision needs, general health and individual goals.

### 1. Phacoemulsification (Phaco)

A 2.8 mm incision is made at the edge of the cornea and a small ultrasonic probe is inserted to break the cloudy lens into fine fragments, which are gently suctioned out through the same opening. An artificial intraocular lens (IOL) is then folded and inserted into the natural lens capsule



**Advantages:**

- Small, stitch-free incision
- Early visual recovery
- Stable wound healing with low infection risk
- Compatible with all premium IOL types

## 2. Micro-Incision Cataract Surgery (MICS)



MICS is an advanced refinement of standard phacoemulsification. At Samyak Drishti, cataract surgery is routinely performed through a micro-incision of approximately 2.0 mm – one of the smallest incisions in modern cataract surgery. The smaller the incision, the less surgically induced astigmatism and the faster the recovery.

**Advantages:**

- Minimally invasive
- Reduced risk of surgically induced astigmatism
- More accurate refractive outcomes
- Faster healing with fewer post-operative restrictions
- Lower risk of wound-related complications
- Compatible with all advanced premium IOLs

## 3. Femtosecond Laser-Assisted Cataract Surgery (FLACS)



Femtosecond laser-assisted cataract surgery uses a computer-guided, bladeless femtosecond laser to perform several of the most delicate steps of cataract surgery with sub-micron precision before the surgeon begins the MICS portion of the procedure.

This results in a more predictable surgical outcome, particularly important when precise IOL positioning is critical.

Steps performed by the laser:

- Image-guided referencing for appropriate planning for the next steps to follow
- Corneal incisions: perfectly sized and positioned, reducing surgically induced astigmatism
- Anterior capsulotomy: a perfectly circular capsular opening for optimal IOL placement
- Lens fragmentation: softens the cataract, reducing the ultrasound energy needed

The surgeon then completes the procedure by MICS.

**Please note, Dr Sujal Shah performs FLACS at Sir H N Reliance Foundation Hospital**

### Why us?

The same femtosecond laser technology that made Dr Sujal a global leader in LASIK and SMILE now enables bladeless, computer-guided cataract surgery.

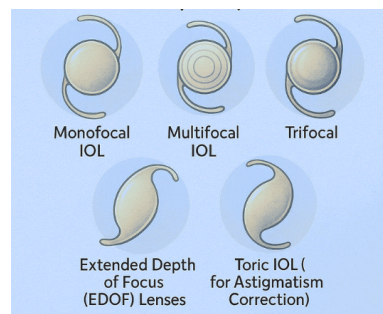
### Advantages:

- Less surgically induced astigmatism and better sealing of incisions after surgery
- **Bladeless & precise:** Highly accurate computer-guided laser performs key steps with sub-micron accuracy, removing manual variability
- **Less energy, less trauma:** Laser pre-fragmentation reduces the ultrasound energy needed inside the eye, meaning gentler surgery and faster healing
- **Superior IOL positioning:** A perfectly circular, capsulotomy maximizes the optical performance of premium Toric, Trifocal and EDOF lenses

**Best suited for patients choosing premium Toric, Trifocal or EDOF IOLs, where precise lens positioning is critical**

## CHOOSING YOUR INTRAOCULAR LENS (IOL)

During cataract surgery, your opaque natural lens is replaced with a clear artificial IOL. Because this is a once-in-a-lifetime decision, Dr. Sujal and Dr. Manisha will take the time to understand your vision goals, lifestyle, and any pre-existing conditions before recommending the most suitable lens.



## Standard (Monofocal) IOLs

### MONOFOCAL IOL

Corrects vision for one distance – usually far. You will need reading glasses for intermediate and near tasks (computer, tablets, mobile, books, menus, newspapers). Good clarity for the chosen distance. Well-suited for patients comfortable wearing spectacles for daily activities.

Monofocal IOLs do not correct astigmatism. If you have significant astigmatism, you may still need spectacles for distance as well.

## Premium IOLs

Premium IOLs are designed to reduce dependence on spectacles after surgery.

### MONOFOCAL Plus IOL

Corrects vision for distance AND also allows significantly enhanced intermediate vision to make everyday tasks easier compared to monofocal IOLs. You will need reading glasses for near tasks (mobile, books, menus). Excellent clarity for the chosen distance. Well-suited for patients comfortable wearing spectacles for part of daily activities.

## Presbyopia Correcting IOLs

Presbyopia correcting IOLs are designed to reduce dependence on spectacles for distance and near vision. They fall into the following categories:

### TRIFOCAL IOL

Three focal zones: distance, intermediate, and near. Provides excellent vision across all distances for most daily tasks – driving, computer work, and reading. Best for patients motivated for reduced dependence on spectacles for most tasks. More than 93% of patients rarely need to use spectacles after surgery. Halos and night glare are significantly reduced compared to bifocal lenses. These are more noticeable during the initial weeks or months, until your brain “adapts” to the new vision. Not ideal for frequent “night drivers”.



### EDOF IOL (Extended Depth of Focus)

Instead of separate focal zones, EDOF IOLs create one continuous elongated focal range – delivering seamless, natural-looking vision from far through to intermediate, with functional near vision at arm-length (car dashboard, desktop computers, laptops, tablets). To see a mobile phone, one may increase the font size and hold at arms length. Extremely low incidence of halos and glare – suitable for frequent night drivers. Fine print (news papers, print reading) may still need reading glasses. Often combined with a trifocal lens in the other eye for full near vision coverage.

### TORIC IOL

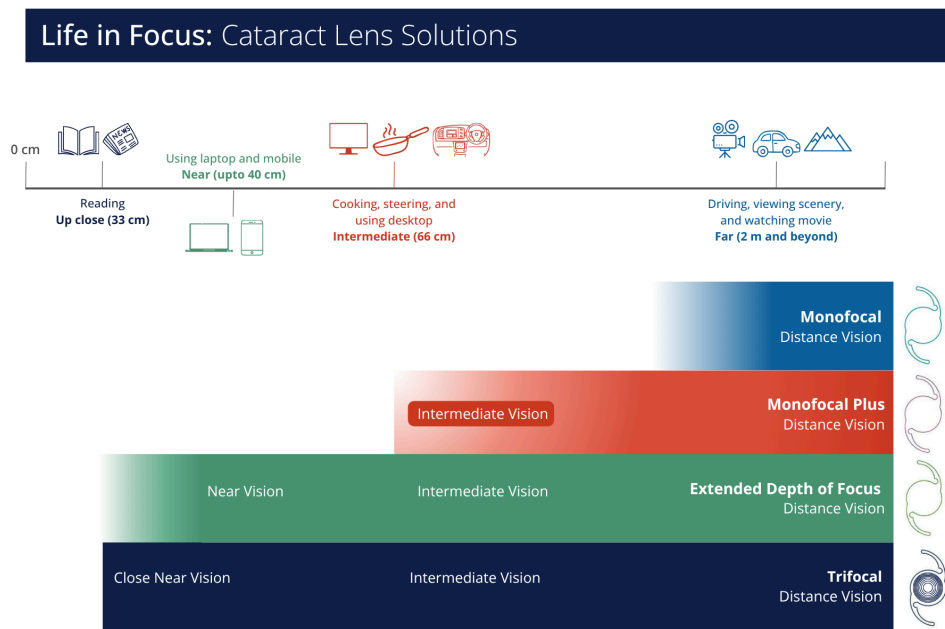
Corrects for pre-existing corneal astigmatism or “cylindrical number” to improve focus. Available in monofocal, advanced monofocal, trifocal and EDOF versions. The need for toric IOLs of any design depends on the ocular biometry and the degree of corneal astigmatism.

## Which Lens Is Right for You?

| Your Goal                                                                           | Recommended IOL Type                                  |
|-------------------------------------------------------------------------------------|-------------------------------------------------------|
| Happy to wear spectacles                                                            | Monofocal IOL                                         |
| Good distance without glasses; ok with glasses for near and some intermediate tasks | Monofocal or Toric IOL                                |
| Minimal glasses for fine distance and near work, night driving is rare              | Trifocal IOL                                          |
| Heavy screen use, regular night driving                                             | EDOF IOL                                              |
| Seamless range + reading fine print, occasional night driving                       | EDOF (one eye) + Trifocal (other eye)                 |
| Have astigmatism                                                                    | Toric IOL-Monofocal, Monofocal Plus, Trifocal or EDOF |



The final IOL recommendation is based on a combination of your biometry results, corneal measurements, eye examination, vision needs and lifestyle preferences.



## WHAT TO EXPECT: BEFORE, DURING & AFTER SURGERY

### Before Surgery

- A detailed pre-operative assessment is performed, including biometry (precise eye measurements for IOL power calculation)
- A general health review and general investigations. You will be asked about current medications. Some blood thinners or prostate medications may need to be paused – your surgeon will advise.
- Antibiotic eye drops are typically prescribed for 1 day before surgery

### On the Day of Surgery

- Do not eat or drink before your procedure (2 hours for patients undergoing “eyedrop or local anesthesia and 6–8 hours for patients undergoing sedation or general anaesthesia).
- Apply the dilating and antibiotic eyedrops to prepare your eyes for surgery
- Please arrive 30 mins before your scheduled time of surgery
- Please bring one responsible adult with you
- Please carry ALL your eye-related or general health reports with you
- Arrange transport, as you will not be able to drive on the day of surgery.

When you arrive at Samyak Drishti, our team will:

- Verify your paperwork, pre-operative investigations, and ensure your pupils are adequately dilated
- Escort you to the surgical suite
- Administer topical anesthetic (numbing eye drops) or, occasionally, a local injection around the eye. You will remain awake and comfortable throughout.

During the procedure, which typically takes 15–20 minutes:

- You may see bright lights and movement, but you will not see any part of the surgery itself.
- You may feel mild pressure as the cataract is emulsified – this is normal and should not be painful.
- Dr. Sujal will talk to you throughout; you are welcome to communicate too.

After surgery you will rest briefly. A staff member will review your post-operative medications and instructions before you go home.

### After Surgery – Recovery

- Most patients notice significantly improved and brighter vision by the very next day.
- Some blurring and fluctuation of vision is normal and settles over the first few days to weeks as the eye adapts to the new lens.
- You will experience mild irritation, dryness, scratchiness, or watering – this is expected and temporary.
- A protective shield is placed over the eye after surgery; wear it when napping or sleeping for the first week.
- You can use protective glasses when outdoors
- Antibiotic, anti-inflammatory, and lubricating eye drops will be prescribed – follow the tapering schedule carefully.
- Dr. Sujal or Dr. Manisha will examine you the day after surgery, and again at scheduled intervals, to monitor healing.
- Most patients resume daily activities within 1–2 days. Complete healing takes approximately 4–6 weeks.

### **Glasses after surgery:**

It takes a few weeks for vision to fully stabilize after cataract surgery. We generally wait until this period has passed before prescribing final reading or distance spectacles.

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## **POST-OPERATIVE CARE INSTRUCTIONS**

Following these guidelines will help ensure the best possible outcome from your surgery:

### **You CAN:**



Resume your regular diet and medication schedule immediately after surgery (after a few hours in case of sedation or general anaesthesia)



Watch TV and read within 24 hours (with the non-surgical eye)



Shower or bathe below the neck from the day after surgery.



Resume light office work and computer use, walking or light exercise in 2–3 days



Wear sunglasses outdoors in bright light



Drive when your Dr. Sujal confirms your vision is adequate, usually after a few days

### **You should AVOID:**



Touching, rubbing, or pressing on the operated eye for at least 1 week



Water, soap, or shampoo to enter the eye for 1 week



Heavy lifting or strenuous exercise for at least 1 week



Swimming, hot tubs, saunas, steam rooms or contact sports (tennis, badminton etc. ) for at least 2 weeks



Eye makeup and eyeliner for 2 weeks (to prevent infection)



Dusty or smoky environments; wear sunglasses outdoors in bright light

#### **Diabetes & Blood Pressure:**

Keeping your blood sugar and blood pressure well controlled in the weeks around surgery significantly aids healing and reduces the risk of complications.

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## **RISKS & COMPLICATIONS**

Cataract surgery is one of the safest and most successful operations in medicine. Serious complications are uncommon, but like any surgical procedure, risks exist. You can discuss these with your surgeon, during the consultation.

### **Possible Complications – Contact Us Immediately If You Notice:**

- Moderate to severe eye or eyebrow pain (mild soreness is normal)
- Significant decrease in vision
- Increasing redness, discharge, or eyelid swelling
- A curtain or shadow coming across your vision
- Sudden increase in flashes of light or a large swarm of new floaters

### **Secondary Cataract (Posterior Capsule Opacification – PCO)**

Weeks, months, or even years after surgery, the back layer of the lens capsule may become cloudy, causing vision to blur again. This is a common and easily treated occurrence called “after cataract” or PCO. It is treated by a brief, painless outpatient laser procedure (YAG laser capsulotomy) that takes about 5 minutes and restores clear vision.

## Contact & Appointments

### **Book Your LASIK Assessment**

*Don't let discomfort become your normal.*

At Samyak Drishti, you will receive a thorough tear film evaluation, a clear diagnosis, and a personalised treatment plan suited to your lifestyle and needs.

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[www.samyakdrishti.com](http://www.samyakdrishti.com)

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### ***Clear Vision. Better Life.***

*This guide is intended for patient education only and does not replace personalised medical advice.*

*Always consult Dr. Sujal Shah or Dr. Manisha Shah for guidance specific to your eyes and health.*